

Stakeholder Analysis for the Hospital Patient Administration System

Prepared for tutor review

The system must balance patients’ need for timely care, clinicians’ need to retain professional decision-making, and administrative and finance teams’ need for traceable operational management. The analysis distinguishes who decides, who performs work and which information or authority passes between teams. The scope covers referral, first appointment, treatment and chemotherapy, payment and refund, patient enquiries, clinic letters and follow-up. It identifies **18 stakeholder groups explicitly named in the case**, plus **one technical-governance role to confirm**.

Power and interest matrix

Keep satisfied High power · lower interest S13 Higher management team S14 Insurers and funding organisations Give concise summaries and involve them for major delay or funding exceptions.	Manage closely High power · high interest S04 Consultants and authorised clinicians S07 Clinical Nurse Specialist Team S11 Finance Team S12 Administrative Management S19 IT and information governance * Confirm decision rules, authority boundaries and acceptance conditions together.
Monitor Lower power · lower interest No confirmed stakeholder in this case. Do not invent a role merely to fill the matrix; reassess if scope changes.	Keep informed and involve in validation Lower power · high interest S01 Patients and representatives; S02 Referrers S03 Medical Secretaries; S05 Outpatient Bookings S06 Treatment Bookings; S08 CNS Administrative Support S09 Call Handling; S10 Pathway Coordinators S15–S18 External service providers Use walkthroughs, prototype reviews and exception-scenario feedback.

Power increases upward ↑ Interest increases to the right →

Scores are analysis judgements, not interview findings. Power means influence over system rules, key authorisation or organisational decisions; interest means the degree of impact and need to participate. Both use a 1–5 scale; 4–5 is high. Lower organisational power does not reduce the importance of a patient’s individual right to consent. * S19 is a derived role.

Stakeholder Register Business and Clinical Users

Scores guide the form of engagement. Clinical authorisation, administrative execution and financial approval must be represented separately in the system.

Stakeholder	Interest and authority boundary	Dependencies and assessment
S01 Patients and authorised representatives External	Interest: Timely care, clear arrangements, communication choice and privacy Role boundary: Provide information, express consent and preferences, and give feedback	Appointments, clinical, finance and communications teams Power / interest: 2 / 5 · Keep informed and involve in validation
S02 GPs and referring organisations External	Interest: Prompt referral processing and complete clinical feedback Role boundary: Submit referrals, provide missing information and receive approved correspondence	Medical Secretaries and Consultants Power / interest: 3 / 4 · Keep informed and involve in validation
S03 Medical Secretaries Internal	Interest: Fewer missing documents and accurate, timely correspondence Role boundary: Check referral documents, request missing information and process approved letters; do not make clinical acceptance decisions	Referrers, Consultants and correspondence provider Power / interest: 3 / 5 · Keep informed and involve in validation
S04 Consultants and authorised clinical professionals Internal	Interest: Patient safety, appropriate treatment and reduced unnecessary recording burden Role boundary: Accept or reject referrals; record consent; authorise treatment, changes and letters; make clinical delay or discharge decisions	Patients, booking teams, CNS and external clinical services Power / interest: 5 / 5 · Manage closely
S05 Outpatient Bookings Team Internal	Interest: Book new and follow-up appointments within requested timeframes Role boundary: Use external scheduling, confirm bookings, record contact outcomes and escalate capacity issues	Consultants, patients, pathway team, scheduling and correspondence services Power / interest: 3 / 5 · Keep informed and involve in validation
S06 Treatment and Chemotherapy Bookings Team Internal	Interest: Coordinate treatment, resources and schedules without duplicate bookings Role boundary: Process authorised treatment requests and changes; await clinical and financial status; do not assess clinical suitability	Clinical team, Finance Team, external treatment and scheduling services Power / interest: 3 / 5 · Keep informed and involve in validation
S07 Clinical Nurse Specialist Team Internal	Interest: Respond promptly to clinical enquiries and provide continuity of support Role boundary: Give authorised clinical advice and support; help define urgent-enquiry rules	Patients, Call Handling Team and Consultants Power / interest: 4 / 5 · Manage closely

Stakeholder Register Coordination and Management

Cross-team handover needs a named owner, status and escalation route. Assumed responsibilities and ratings are identified explicitly.

Stakeholder	Interest and authority boundary	Dependencies and assessment
S08 CNS Administrative Support Team Internal	Interest: Support nursing-team coordination Role boundary: Named in the case; assumed to log and coordinate work pending role confirmation	CNS, Call Handling Team and other administration teams Power / interest: 2 / 4 · Keep informed and involve in validation
S09 Call Handling Team Internal	Interest: Route enquiries accurately and avoid missing urgent concerns Role boundary: Log, classify, prioritise and route enquiries; answer only authorised administrative information	Patients, CNS, Finance and administration teams Power / interest: 2 / 5 · Keep informed and involve in validation
S10 Patient Pathway Coordinators Internal	Interest: Identify pathway blockages and keep letters and appointments on time Role boundary: Monitor outstanding correspondence, issue weekly reminders and escalate delays	Consultants, booking team and Administrative Manager Power / interest: 3 / 5 · Keep informed and involve in validation
S11 Finance Team Internal	Interest: Traceable funding, accurate reconciliation, no duplicate charging and correct refunds Role boundary: Record funding approvals, manage payment exceptions and authorise refunds; do not decide clinical need	Patients, funders, payment provider, clinical and booking teams Power / interest: 4 / 5 · Manage closely
S12 Administrative Management Team and Administrative Manager Internal	Interest: Manage backlogs, improve efficiency and maintain accountability Role boundary: Review reports, manage one-month letter escalation and coordinate process issues	Pathway Coordinators, Consultants, Finance and senior management Power / interest: 5 / 5 · Manage closely
S13 Higher management team Internal	Interest: Resolve major delays and understand service risk Role boundary: Receive three-month escalation and intervene in major process issues	Administrative Manager and clinical leadership Power / interest: 5 / 3 · Keep satisfied
S14 Insurers and funding organisations External	Interest: Clear coverage, amount and limitation information Role boundary: Provide funding approval, reference, approved amount and limits	Finance Team and patients Power / interest: 4 / 3 · Keep satisfied

Stakeholder Register External Services and Governance

External service availability can directly affect the patient pathway. Providers help confirm interfaces and exception handling, while hospital professionals retain their relevant business decisions.

Stakeholder	Interest and authority boundary	Dependencies and assessment
S15 External scheduling service provider External	Interest: Accurate exchange of availability and appointment data Role boundary: Provide scheduling capability and results; confirm interface and outage responsibilities	Outpatient and Treatment Bookings teams Power / interest: 3 / 4 · Keep informed and involve in validation
S16 External treatment, laboratory and imaging providers External	Interest: Available resources and correctly matched clinical results Role boundary: Provide service availability and treatment or test information	Clinical team and Treatment Bookings Team Power / interest: 3 / 4 · Keep informed and involve in validation
S17 External correspondence service provider External	Interest: Accurate delivery of approved letters and traceable sending outcomes Role boundary: Distribute correspondence to approved recipients and channels; do not approve clinical content	Medical Secretaries, booking team and recipients Power / interest: 2 / 4 · Keep informed and involve in validation
S18 Payment Service Provider External	Interest: Accurate payment and refund processing with reliable transaction status Role boundary: Process payments and approved refunds; return reference, date, amount and status	Finance Team, patients and hospital system Power / interest: 3 / 4 · Keep informed and involve in validation
S19 IT and information governance leads To confirm	Interest: Secure identification matching, access control, auditability and continuity Role boundary: Assumed to confirm technical and information-governance requirements; roles and authority remain to confirm	All business teams and external providers Power / interest: 4 / 4 · Manage closely

Engagement approach

Audience	Suggested engagement	Evidence for review
High-power, high-interest roles	Run decision-rule and authority walkthroughs; reconfirm key rules when they change.	Confirmed clinical, administrative and financial authority map.
High-power, lower-routine-interest roles	Provide operational and exception summaries; trigger engagement for major delay or funding issues.	Escalation decisions, funding constraints and unresolved issues.
Patients and routine business users	Use appointment-notice, enquiry-routing and rescheduling prototypes for feedback.	Evidence on understandability, accessibility and workflow completeness.
External providers and technical governance	Confirm message fields, status, duplicate requests, failure and recovery scenarios.	Interface responsibilities, correlation identifiers and exception handling.

These are recommended engagement methods. No real stakeholder interviews or approvals are claimed in this analysis.

Dependencies and Decision Authority

How to read this table: arrows show the principal business handover; “↔” means two-way exchange of status. Dependencies include information, authority and external-service results.

Dependency	Stakeholder relationship	Exchange	Control requirement and source
D01 Referral review	Medical Secretaries → Consultant	Checked referral documents → accept, reject, request more information or redirect decision	Only a Consultant accepts; no first appointment before acceptance. ¶4–5
D02 First and follow-up appointments	Clinician → Outpatient Bookings → scheduling and correspondence providers	Speciality, priority, timeframe and patient requirements → booking and contact outcome	Appointments within two weeks also require telephone contact; unavailable slots are escalated. ¶6–7, ¶25
D03 Treatment scheduling	Clinician → Treatment Bookings; Finance → Treatment Bookings	Authorised request, consent, resource needs and funding or payment status	Clinical authorisation and applicable funding conditions must be met; urgent care has a recorded clinical exception. ¶8–13
D04 Cycle continuation and change	External clinical services → Clinician → Treatment Bookings and Finance	Test or review results → clinical continue, delay or modify decision	Administrative staff do not assess fitness; changes need formal authority and financial review. ¶15–16
D05 Payment and refund	Finance ↔ payment provider; Finance ↔ funder	Transaction reference, amount, date and status; funding approval and limits	Investigate a missing response after successful collection; authorised Finance staff approve refunds. ¶10–14
D06 Patient enquiry	Patient → Call Handling → CNS / Finance / administration	Classification, priority, assigned team and outcome	Clinical matters go to qualified clinicians; urgency rules remain to confirm. ¶17–19
D07 Letters and delayed-letter escalation	Consultant → Medical Secretaries → correspondence provider; Pathway Coordinator → management	Approved letter, recipient and send record; outstanding items and reasons	Consultant owns clinical content; coordinator reminds weekly and escalates by timeframe. ¶20–24

Three decision types must remain separately authorised

Clinical decisions

Consultants and appropriately authorised clinicians decide referral acceptance, treatment necessity, fitness and clinical change.

Administrative execution

Secretaries and booking teams check documents and arrange bookings or correspondence. They return suspected clinical errors for review.

Financial decisions

Authorised Finance staff decide funding, payment investigation and refund approval. They do not decide clinical necessity.

Urgent treatment is not approved by Finance: clinicians may authorise proceeding when delay would create patient risk, record the reason and refer it to Finance for resolution. ¶13–14

Stakeholder Conflicts and System Requirements

The following conflicts are either explicit in the case or derived from stakeholder dependencies. The proposed responses support classroom discussion and remain subject to stakeholder confirmation.

Conflict	Stakeholders	Why it conflicts	Proposed response and source
C01 Care time and audit completeness	Clinical team ↔ administration and finance management	More recording improves traceability but can add to clinical workload	Capture actor and time automatically; require only necessary fields; review form burden together. ¶27–29
C02 Urgent treatment and advance-payment condition	Patients and clinical team ↔ Finance	Waiting for payment confirmation may conflict with timely care	Retain the recorded clinical exception and hand the case to Finance for follow-up. ¶12–13
C03 Clinical timeframe and capacity	Clinical staff and patients ↔ booking teams and external services	A clinically requested timeframe may have no available slot	Keep the booking pending, notify the responsible team and escalate; do not silently book outside the timeframe. ¶7, ¶9, ¶25
C04 Information sharing and access boundaries	All business teams ↔ patients	Effective coordination needs information sharing; patients expect privacy	Role-based minimum access, access/change audit trail and payment-data minimisation. ¶11, ¶27–30
C05 Standard communication and individual needs	Patients ↔ booking and secretarial teams	Standard channels are easier to manage; some patients need post, translation or accessible format	Record communication preference and support needs; meet letter and telephone requirements. ¶6, ¶30
C06 Rapid change and formal authorisation	Clinical team ↔ Treatment Bookings and Finance	An urgent pause must be rapid; routine changes require a full approval record	Use formal modification requests normally; record and authorise safety-related urgent pauses afterwards, then assess financial impact. ¶16

From stakeholder need to observable system behaviour

Requirement	Stakeholders	What should be observable when the system is checked
R01 Role and authority control	S03–S12, S19	A referral is not bookable until Consultant acceptance; administrative roles are blocked from unauthorised approval.
R02 Booking and contact traceability	S01, S05–S06, S10, S15	Every contact attempt and outcome is recorded; near-term appointments generate phone contact; unavailable slots have an owner.
R03 External-service resilience and de-duplication	S06, S11, S15–S18	An outage leaves the work pending; repeat booking or payment status messages do not produce duplicate outcomes.
R04 Funding, payment and refund traceability	S01, S11, S14, S18	A missing payment response creates an investigation; refunds need Finance authority; payment stays linked to booking and refund.
R05 Letter and escalation monitoring	S03–S04, S10, S12–S13	Letters incomplete or unapproved after seven days are delayed; weekly reminders and one-/three-month escalation are applied; reminders stop on completion.
R06 Enquiry, privacy and continuity	S01, S07–S09, S19	Enquiries have classification and a responsible team; clinical matters route safely; access is auditable and downtime work has a recording procedure.

This is a requirements-traceability summary. The behaviours are proposed acceptance conditions, not a claim that the system is built or tested.

Open Questions and Presentation Notes

This document is a desk analysis of the supplied course case. It does not claim hospital access or real-user interviews. Confirm the questions below with the tutor acting as client, then update ratings, responsibilities and requirements.

Identifier and subject	Current basis or assumption	Consult	Question for the tutor
A01 CNS Administrative Support responsibilities	The case names the team but gives no detailed workflow; it is currently treated as work-logging and coordination support.	S07, S08	Which tasks does the team perform? Which fields may it view or amend?
A02 Management and funder interest	Senior management and funders are rated high-power but lower routine interest; reconsider on a major event or funding dispute.	S13, S14	Who is the actual project sponsor? Who attends routine reviews? Who gives final funding confirmation?
A03 Technical and information-governance role	This role is derived from access, audit and availability requirements; it is not named in the case.	S12, role to confirm	Which roles own technical operations, information governance and approval of related requirements?
Q01 Urgent enquiry and clinical exception	The case says urgent-enquiry rules are incomplete; no clinical threshold has been invented.	S04, S07, S09, S11	Who determines urgency? Who records urgent treatment and subsequent authorisation, and by when?
Q02 Time and escalation boundaries	The analysis retains “seven days, weekly, more than one month, more than three months”; it does not convert them to working days or 30/90 days.	S03, S04, S10, S12	Are these calendar days? When does the month begin? How are reminders handled after escalation?
Q03 Funding and external-service failure	The case sets principles for funding, payment and service exception but not detailed rules.	S06, S11, S14–S18	Which cases require advance payment? Who approves exemption? What are reconciliation, retry and recovery rules?
Q04 Patient representatives and communication needs	The case supports representatives and accessible communication but does not define verification or authority scope.	S01, S03, S05, S09	How is representative identity and authority verified? How are translation, accessibility and failed contact handled?

Short presentation script

“We identified 18 stakeholder groups explicitly present in the patient pathway, from referral to follow-up, and one technical governance role that needs confirmation. Consultants, Clinical Nurse Specialists, Finance and Administrative Management have strong influence over key rules and need close involvement. Patients have less organisational system power but are most affected, so they must validate notices, appointments and communication experience. The main design point is to keep clinical decisions, administrative execution and financial approval separate while connecting teams through formal handovers, exception handling and escalation. We need the tutor to confirm the urgent-enquiry rules, CNS administrative support responsibilities and timing boundaries.”

Source and reference method

Course case: Case Study — Hospital Patient Referral, Treatment and Administration System, supplied course document, undated.

¶n refers to the sequence of non-empty top-level body paragraphs in the source document; the title is paragraph 1. Main locations: ¶4–9 referral and booking; ¶10–16 funding, treatment and modification; ¶17–19 enquiries; ¶20–26 letters, follow-up and non-attendance; ¶27–30 access, audit, operations and patient expectations; ¶31 asks for stakeholder analysis and documented assumptions. Ratings, matrix placement, engagement method and proposed responses are this analysis.